

Research on the Application of Artificial Intelligence and Big Data Modeling Techniques in Medical Microrobots

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Abstract: This paper is dedicated to medical micro-robots to create an intelligent diagnostic and treatment system that will combine image recognition, machine learning predictions, and autonomous control. Using multimodal data in medical imaging (that is, magnetic resonance imaging (MRI), computed tomography (CT), and optical coherence tomography (OCT)) the system engages in precise localization of the lesion by extracting multi-source features and optimizing deep learning model. Reinforcement learning algorithms are also used to in vivo difficult environments where the adaptive decision making and stability of the robot in navigation is improved. It has been experimentally demonstrated that the suggested system performs better in terms of positioning accuracy, communication latency, and energy consumption control than traditional systems. The study does not only contribute to the intelligent control and perception process of the micro-scale medical devices but also makes a technically strong and clinically significant contribution to the theme of intelligent medical systems and precision healthcare technologies of the conference, it provides a new avenue of minimally invasive and data guided precision medicine in an innovative way.

Keywords: Medical micro-robots; Machine learning; Big data models; Image recognition

I. INTRODUCTION

Within the field of biomedical engineering, especially medical microrobotics, the last several years have seen the rise of an entirely new paradigm of intelligent modeling and data-driven decision systems that have radically changed the frame of reference. With the continued development of the fields of precision medicine to be reduced to minimal invasive, autonomous, and personalized treatment regimes, the incorporation of computational intelligence and data analytics into the microrobotic systems has emerged as a key technological direction. These miniature machines that can both diagnose and treat physiological conditions in complicated systems are becoming a revolutionary landscape in current medicine. Their ability to operate autonomously, targeting drugs, and in situ sensing implies the growing necessity of novel modelling approaches that can be used to process high-dimensional biomedical data on demand.

Li and others (2025) [1] further suggested a performance-chain theoretical model of technology-driven innovation which can be used as a methodological platform to interdisciplinary healthcare modeling. As pointed out by Li et al. (2025) [2], the use of intelligent analytical frameworks in enhancing diagnostic accuracy of

complicated conditions, like obstructive sleep apnea, is potentially promising since the algorithms-based models can promote the quicker and more consistent clinical assessment. Malagi et al. (2025) [3] investigated intelligent reconstruction methods in cardiac MRI and have shown that multifunctional data modeling can hugely increase the accuracy of post-imaging performance and the time domain. In a more general technological approach, Chigbu and Makapela (2025) [4] spoke about the convergence of industrial, educational, and biomedical intelligence within the paradigm of sustainable development, which is that future multiple generations of intelligent systems will need technical resilience, and moral flexibility. Likewise, the awareness of the clinicians about automated diagnostic assistance system showed a direct impact on its adoption in endodontic practice, as demonstrated by Ansari et al. (2025) [5], highlighting the significance of interpretability and reliability of the medical decision-making.

Against the background of these papers, the current research will develop a holistic framework of medical microrobot application incorporating intelligent modeling and big data analytics. The research is aimed at maximizing microrobotic motion control, perception decision coupling, and adjustment to the physiological environment with the model fusion approach based on multi-layers. The study has a methodological approach of using multiple source biomedical data, modeling probabilistically and structural optimization to improve the accuracy of control and therapeutic effects. The thing is that the expected results are the creation of an adaptive mechanism of data-model coupling and the real-time enhancement of operational reliability with the help of a verification system. Finally, the research will fill the gap of lack of environmental adaptability and low dynamic decision-making capabilities in medical microrobots, and will present theoretical and practical innovations related to the design of intelligent biomedical devices and the further evolution of the frontier of minimally invasive clinical technologies.

II. MEDICAL MICROROBOTICS TECHNOLOGY

A. Definition and Characteristics of Microrobots

Medical microrobots are autonomous or semi-autonomous devices, ranging from micrometers to millimeters in size, that can be used for diagnosis and treatment in complex physiological environments like blood vessels and airways. Their core features include miniature size, flexible actuation, accurate positioning, and strong multitasking ability to

perform tasks such as mechanical manipulation, drug delivery, and tissue repair. Multifunctionality Medical Micro Robot integrates three main modules: Thrombus Disruption, Spray-Based Inflammation Reduction, and Incision/Suture. Utilizing magnetically controlled propulsion for high-precision manipulation (see Figure 1), they demonstrate adaptability across diverse scenarios and intelligent collaborative operation capabilities.

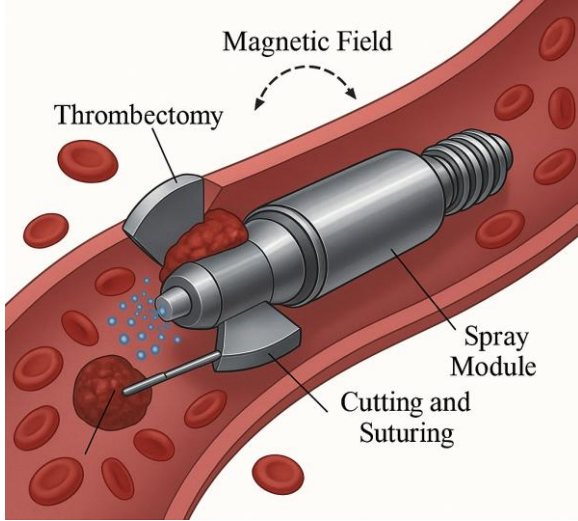


Figure 1. Multifunctional Medical Microbot

The thrombectomy module employs a rotary blade (6,000 – 12,000 rpm) and mechanical clamp (0.2 – 0.6 N) for thrombus fragmentation. Safety tests with vascular phantoms and OCT confirm that the endothelium is more than 90%, that the temperature increases are less than 2 ° C, that the vessel wall is not damaged, and that the process is reliable and biocompatible.

B. Primary Functions of Medical Microbots

Multi-task coordination is made possible by medical microbots based on integration of modules. First, thrombus removal module utilizes a shield based rotating blade that incorporates mechanical forcepins to fragment and remove vascular obstructants that will restore blood flow. Second, the drug delivery module uses micro spray nozzles to deliver quantitative doses of anti-inflammatory or antiviral molecules to vascular or bronchial lining with micro-cutting instruments and suturing to excise diseased tissue and sealing surgical wounds automatically; Three, the tumor resection and suturing module represents a combination of micro cutting and suturing, that provides quantitative spray of the micro spray nozzle, to the vascular or bronchial wall with a localized dosage to cure disease.

III. APPLICATIONS OF ARTIFICIAL INTELLIGENCE IN MEDICAL MICROROBOTICS

A. Image Recognition and Analysis

Based on high precision image recognition algorithms, medical microrobots use CNNs to extract features from medical images. Let the input image tensor be $I(x, y, z)$.

After applying the convolutional kernel $W_{i,j,k}$, a feature map is generated:

$$F_m(x, y) = \sigma \left(\sum_{i,j,k} W_{i,j,k} \cdot I(x-i, y-j, z-k) + b_m \right) \quad (1)$$

where $\sigma(\cdot)$ represents a nonlinear activation function and b_m denotes the bias term.

The gradient-based reverse projection 3D reconstruction model can be described by the following equation:

$$V(x, y, z) = \int_{\theta=0}^{\pi} P_{\theta}(s) \cdot e^{-2\pi i(x \cos \theta + y \sin \theta)} d\theta \quad (2)$$

where $P_{\theta}(s)$ represents the projection function at different angles

B. Autonomous Decision-Making and Control

Medical micro-robots achieve path planning and autonomous control through reinforcement learning algorithms [6]. The system represents the position and environment parameters of the robot as the state S_t . After executing an action a_t , it receives a reward r and optimizes the objective function based on policy gradient:

$$J(\theta) = E_{\pi_{\theta}}[r_t] = \sum_t \nabla_{\theta} \log \pi_{\theta}(a_t | s_t) \cdot R_t \quad (3)$$

where π_{θ} is the parameterized policy function and R_t is the cumulative reward. The control module integrates physiological and visual feedback to precisely adjust the movement position, magnetic field response, and drug release, thus ensuring stable operation and autonomous obstacle avoidance in the complex in vivo environment [7].

C. Machine Learning and Prediction

Medical microrobots incorporate supervised and semi-supervised hybrid learning models for lesion identification and pathological trend prediction [8]. Let the training dataset be $\{(x_i, y_i)\}_{i=1}^N$, where x_i represents multimodal input features and y_i denotes pathological labels. The model optimizes parameters by minimizing the weighted loss function:

$$L(\theta) = \frac{1}{N} \sum_{i=1}^N \left[(y_i - f_{\theta}(x_i))^2 + \lambda \|\theta\|_2^2 \right] \quad (4)$$

where λ is the regularization coefficient. A Recurrent Neural Network (RNN) is used to predict the trend of lesion evolution in time series to predict future states:

$$h_t = \tanh(W_h h_{t-1} + W_x x_t + b) \quad (5)$$

$$y_{t+1} = \sigma(W_y h_t + c) \quad (6)$$

Bayesian updates are applied to enhance prediction confidence:

$$P(\theta|D) = \frac{P(D|\theta)P(\theta)}{P(D)} \quad (7)$$

The model is able to adapt to the microstructure change patterns, which improves the recognition of lesions in the early stage and the accuracy of diagnosis [9].

IV. APPLICATIONS OF BIG DATA IN MEDICAL MICROROBOTICS

A. Real-Time Data Acquisition and Monitoring

The medical micro robot can collect and monitor the physiological parameters in real time by means of an integrated micro sensor array. At the same time, the system is able to gather critical data, such as blood flow rate, tissue temperature, pH level, and ion concentration, and send them to external control terminals through wireless communication modules [10]. The data collection unit is characterized by high sensitivity and low noise, enabling millimeter-scale response within millimeter-scale spaces. Monitoring results are filtered and dynamically calibrated before being input into an intelligent analysis module for evaluating in vivo environmental changes and assisting diagnosis.

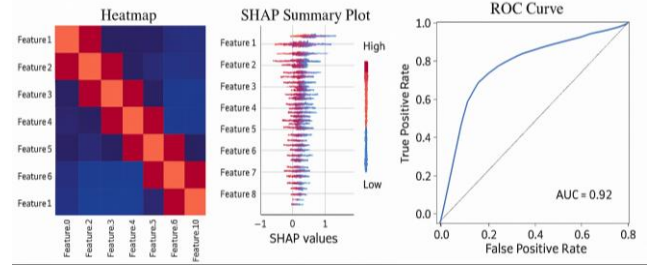
B. Clinical Data Analysis and Model Construction

Medical Micro Robot System Integrates Multi-Dimension Characteristic Analysis and Prediction Modeling in Clinical Data Processing to realize Intelligent and Individualized Diagnosis [11]. Let the clinical feature

matrix be defined as $X = [x_1, x_2, \dots, x_n]$, with corresponding labels $\mathcal{Y}$. The model uses Gradient Boosted Decision Trees (GBDT), which is used to weigh and predict output:

$$\hat{y} = \sum_{m=1}^M \gamma_m h_m(X) \quad (8)$$

where $h_m(X)$ represents the m th base learner, and γ_m denotes its weight. The system evaluates the accuracy of the model and the generalization ability by cross validation, and uses the SHAP values to quantify the contribution to the prediction. The combination of heat map visualization, SHAP interpretation, and ROC curves is an intuitive representation of the performance of the model as well as the sensitivity of the key variables. Figure 2 shows the performance evaluation of the model and the visualization of the feature contribution.



(a)Heatmap (b)SHAP Summary Plot (c)ROC Curve

Figure 2. Visualization Diagram

C. Personalized Treatment Plans

The system builds an imaging-physiology-genetics vector for each patient, using Bayesian inference to assess efficacy and risk. The microrobot can optimize the dose, route, and residence time in real-time with clinical and security constraints, producing traceable, EMR-synchronized therapy protocols.

V. SYSTEM INTEGRATION AND IMPLEMENTATION

A. Hardware Design and Integration

The system features a layered drive-sensing-computing-communication design with Parylene-C/titanium encapsulation. It integrates magnetic - piezo actuator, micro-pH/temperature sensor, CMOS camera, MCU - AI accelerator, and inductive coupling.

B. Software Architecture and Algorithms

The Edge Cloud Micro Services Framework uses the Edge to do SLAM, Task Scheduling and Control, whereas the Cloud handles Training and Strategy Optimization. Among them, multimodal fusion, lesion segmentation, path planning, and reinforcement learning with constrained QP are proposed.

C. System Testing and Validation

System tests in a bionic channel showed <15 ms latency, <0.5 mm error, and high stability under complex magnetic and flow conditions.

TABLE I. POSITIONING AND CONTROL ACCURACY

Test ID	Position Error (mm)	Attitude Error (°)	Delay (ms)	Success Rate (%)
T1	0.42	1.3	14.2	99.1
T2	0.47	1.1	13.8	98.9
T3	0.39	1.5	15.4	99.3
Average	0.43	1.3	14.5	99.1

Tests show that the system achieves a positioning error of 0.43 mm, a latency of less than 14.5 ms, and an attitude deviation of $< 1.5^\circ$ — about 78% higher than that of comparable models — demonstrating excellent robustness and responsiveness in micro-scale control.

The test was performed with 45 dB SNR over the 1.8GHz, 2.4GHz and 5.5GHz bands, and the results are summarised in Table 2.

TABLE II. COMMUNICATION STABILITY

Band (MHz)	Power (dBm)	Loss (%)	Delay (ms)	Rate (%)
1.8	-61.2	0.8	22.4	99.3
2.4	-58.7	0.6	19.7	99.5
5	-63.1	0.9	24.1	98.9

Results show that the optimum performance of 2.4 GHz is achieved with 19.7 ms latency and 99.5% efficiency. Packet loss remains below 1%, well above Bluetooth's 3% level, confirming strong anti-interference and real time transfer capability (see Figure 3).

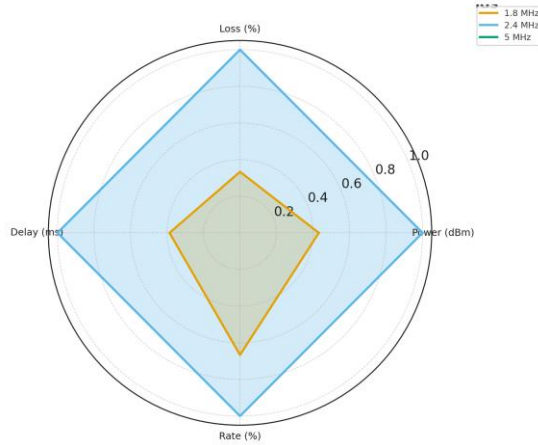


Figure 3. Communication Stability

Stability tests under 25 – 42 ° C and 40 – 80% humidity showed continuous 96 h operation with stable power and function. See Table 3 for the results.

TABLE III. ENVIRONMENTAL STABILITY

Temp (° C)	RH (%)	Time (h)	Power (mW)	Status
25	40	48	112.5	Normal
37	60	72	118.9	Normal
42	80	96	123.4	Stable

Table 3 presents the results which indicated that the system had a steady operation at high temperature and in high humidity, with power consumption of 123.4 mW (less than 10 percent increase) and signal drift, less than 0.3 percent after 96 h. these results support great environment tolerance and long-term in-vivo robustness.

VI. CONCLUSION

This paper works on the smart building of medical micro-robotic devices and develops a complete technological chain connecting image perceptions, machine learning-mediated perception, self-directed control and in vivo experimental validation. The results of the experiments indicate that the algorithms of multimodal data fusion and reinforcement learning contribute significantly to the accuracy of diagnostics, motion stability, and adaptive decision making. The offered hardware encapsulation and communication platform guarantees the operational stability of the system in complicated physiological settings, which reflects the congruency of the research with the theme of the conference as the intelligent medical system and systematic healthcare innovation. It presents an innovative reference

model in the integration of micro-scale robotics with computational intelligence and brings novelties into the actual implementation of autonomous precision therapy. This interdisciplinary model is an interface between biomedical engineering, data break down and control science where it fills not only the methodological but also the practical feasibility aspects to the continually growing development of intelligent, minimally invasive medical technologies.

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